



Beyond border health: Infrastructural violence and the health of border abolition[☆]

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ABSTRACT

Most existing approaches to border health focus on identifying the social determinants that produce ill health and health disparities among migrants, including language barriers, documentation status, and trauma associated with migration. Attention to these kinds of problems can lead to policy and clinical changes that indeed help improve quantitatively measurable outcomes for patients. However, these approaches usually ignore the larger historical and political framework that determines the determinants – the underlying infrastructure of ill health, or what we term the infrastructural determinants of health. In this paper, we outline specific infrastructures involving race, political economy, history, and most importantly, borders themselves, that lay the foundations for border illness. We examine the plans, histories, policies, and peoples involved in building the conditions for migration, particularly out of the Northern Triangle, including forces of colonialism, US imperialism, neoliberalism, and border militarization. In place of a tacit acceptance of the modern system of borders, we argue for border abolition as a vital but underused treatment in the repertoire of medical intervention. Outlining the rights of people to stay and to move, and drawing on lessons from the prison abolition movement, we offer policies and practices towards a ‘no borders’ system that privileges liberatory solidarity with migrants by explicitly challenging global infrastructures that drive displacement. In doing so, we offer an emergent framework for a medical border abolition that treats both the causes and symptoms of a widespread global sickness.

Something was off. Sandra’s 6-year old son Isaac wouldn’t make full eye contact. His feet rhythmically bounced off the bed where he sat silent. The chief complaint was constipation for three days. The typical questions were elicited. Sandra was concerned but the exam was fairly benign. The child was initially stoic but then became incensed during the exam, most likely secondary to anxiety since his abdomen was soft and non-tender. It wasn’t until just prior to discharge that the nurse noticed Sandra was wearing an ankle bracelet. It was only then, upon gentle questioning - discharge paperwork in hand - that bits of her story came out. The abuse by her partner in El Salvador, a member of the MS-13. The days he locked her in his home; beatings with tree branches, lamps, a blender. Every household item became a weapon. The time he waterboarded her in a sink in front of Isaac. How she was severely

beaten and gang raped when she tried to leave. The days, months, years of this that eventually led her to flee the only place she had ever known and her entire family. The time she was robbed at gunpoint on the journey north or how she was held for four months at the southern Mexico border, only to be raped again by a police officer in exchange for keeping her papers.

Aged 23, she arrived in Tijuana, still hopeful, crossing the border on foot. She was so overjoyed to see the border guard, who she saw as a beacon of safety, that she smiled for the first time in over a year at the sight of him and said “*buenos días*.” He responded “not a good day for me now that you’ve arrived.” He put her in a detention facility where Isaac got sick. He developed a high fever and he stopped eating. She begged to see a doctor but they told her “*no hay* (there is none)” and so she stroked

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his head and mopped away the sweat. By that time she realized she was pregnant. A few days later she was released back to Tijuana under the “Migrant Protection Protocols” (MPP)ⁱ where she’d live for months in an overcrowded shelter, shuttled back and forth between the detention and court for her asylum proceedings. Each time, Isaac wouldn’t eat for days. There was no school for him, only the occasional US volunteers who’d bring books. He got chickenpox and the mumps. But she was lucky, she says. She met a lawyer one day at the shelter who took on her case - one of the 1% who would ever have one. She gave birth in Tijuana and since her child didn’t qualify for MPPⁱⁱ, she was released with an ankle bracelet into the US to await her next court date, but only after “processing,” which meant two weeks in detention. Isaac didn’t talk for three days after release.ⁱⁱⁱ

1. Beyond border health: toward infrastructural determinants of health

Treating migrants like Sandra and her son Isaac has become a part of clinical realities for many providers across the United States engaged in the broad practice of border health. Most of these encounters are treated as standard biomedical problems without any kind of digging into the deeper conditions of detention, migration, or domestic abuse (Speed, 2019). In rare cases that these underlying social pathologies are even identified, often by very well-intentioned and caring clinicians, the most common response to the patient is, in our experience, shaped by a humanitarian or human rights framework. Some have pushed toward work on the social determinants of health, advocating for housing security, citizenship, and other necessities as healthcare (Bourgois et al., 2017; Farmer, 2003; Link and Phelan, 1995; Marmot, 2005; Marmot and Wilkinson, 2005; WHO Commission on Social Determinants of Health, 2008).^{iv} Seeing a patient like Sandra as a victim of domestic abuse and as a precarious citizen, a humanitarian clinician might refer her to a women’s shelter or offer medical expertise towards a legal asylum claim. But even this much-needed attention to social determinants usually takes for granted the larger historical and political framework and their physical manifestations that determines the determinants - the underlying infrastructure of ill health, or what we term the infrastructural determinants of health.

Many existing frameworks on border health focus on identifying the social contexts that produce ill health and health disparities among migrants (Castañeda et al., 2015). Among the many subjects that receive attention are: obstacles to accessing care, including undocumented status and language barriers (Almeida et al., 2013; Hacker et al., 2015; Ku and Matani, 2001); trauma and violence associated with migration (Cleary et al., 2018; Keller et al., 2017; Torres et al., 2018); structural racism (Gee and Ford, 2011; Philbin et al., 2018; Viruell-Fuentes et al., 2012); the lack of legal and civil protections (Samra et al., 2019; Bustamante et al., 2012; Ferrer, 2019); and occupational injuries associated with manual labor jobs, particularly among migrant farm workers

(Holmes, 2013; Moyce and Schenker, 2018; Seymour et al., 2018). These are, no doubt, important primary considerations for biomedical doctors trying to establish a basis for understanding illness beyond the immediately presenting biopathogenic process. Attention to these kinds of problems can lead to policy and clinical changes that indeed help improve quantitatively measurable outcomes. Similarly, structural violence offers another useful framework of analysis by recognizing the complex web of social, political, and economic forces that constrain the potential of individual and community health. Yet, as has been noted by others, the ambiguity of structural violence makes it both difficult to assign specific responsibility for suffering and to operationalize interventions to undo the “social machinery of oppression” (Farmer et al., 2004; Rodgers and O’Neill, 2012).

In our view, many of these approaches shy away from or ignore important politico-economic histories and policies that constitute some of the more obscured or forgotten determinants of health - the infrastructural ones. Addressing social contexts and determinants only begins to scratch the surface in confronting and transforming an enormous set of colonizing, racist hegemonic assumptions and ways of thinking - a project necessary in truly revolutionizing the ways in which migrant illness is approached and treated. Eschewing humanitarian and human rights approaches to border health, and responding to Castañeda et al. (2015)’s call for attention to the historical and political economic contexts producing migration, we argue for a root cause approach that not only identifies and addresses the social determinants of health but also creates new possibilities for thinking what constitutes the ‘social’ and ‘health’. Drawing on recent conceptualization of ‘infrastructural violence’ (Rodgers and O’Neill, 2012), we call this kind of excavation an ‘infrastructural determinant of health.’ By this, we refer to the specific elemental ways in which systemic or structural forms of illness such as racism, documentation, and occupational injury are produced and by whom. In other words, we call attention to the concrete practices that lay the groundwork for the injurious structural determinants of health. We propose ‘infrastructural determinants of health’ as a way of nuancing work done by similar concepts such as fundamental causes, social determinants of health, structural vulnerabilities, and structural violence in two ways.

First, the concept of ‘infrastructural determinants of health’ recognizes that many of the existing and visible social or structural determinants of health (racism, citizenship, housing, climate change, and so forth) themselves have complex histories with even deeper roots - and focuses on the specific elements comprising the structure rather than the often intimidatingly mystical structure itself. In practice, this means identifying the precise elements constituting the vagaries of social or structural determinants such as social support, unemployment, and addiction, among other forces. Second, we find that infrastructural determinants of health are more useful in concretely identifying the responsibility for suffering than more amorphous notions of ‘structural’ or ‘social’ determinants of health, or ‘structural violence’ (Ferguson, 2012). If examining ‘social determinants of health’ prevent us from reducing ill health to a technical issue (Marmot, 2005), then contemplating ‘infrastructural determinants of health’ prevent us from reducing social violence to faceless and seemingly insurmountable structures or processes (e.g. ‘financial insecurity’ or ‘homelessness’) which makes poverty or violence seem to appear out of thin air rather than as the direct result of underlying infrastructural mechanisms.

In contrast to “faceless set of fleeting social connections”, infrastructural determinants of health and infrastructural violence reference the lasting material channel, in this case borders and walls, of structural violence and “provides an ideal space for reflecting upon the systemic forms of violence that occur through a society’s effort to govern itself” (Rodgers and O’Neill, 2012). The border, like prison walls, water pipes, and sewage systems, is a physical infrastructure but also a political and ideological infrastructure, “critical both to differentiated experiences of everyday life and to expectations of the future” (Anand et al., 2018). Centering infrastructure as a critical and often overlooked determinant

ⁱ The Migrant Protection Protocols, also known as the “Remain in Mexico” program, are a series of US government policies that allow the US border officials to return individuals and families from certain Central American countries (excluding Mexico) to dangerous border cities in Mexico while their asylum claims are adjudicated in US immigration courts.

ⁱⁱ Children born in Mexico while their parent(s) are awaiting asylum proceedings do not qualify for entry into the Migrant Protection Protocol (MPP) as they have Mexican birth certificates.

ⁱⁱⁱ Sandra’s story is a conglomeration of multiple patient encounters that have purposely been combined to protect the identity of any one client, many of whom are still in asylum proceedings. The commentary in this article is partially based on personal experiences providing care at the US-Mexico border.

^{iv} This distinction between ‘economic migrants’ and ‘refugees’ is a major problem in establishing the so-called ‘veracity’ of asylum seekers - a distinction made on particular and problematic humanitarian visions of justice that reproduce an exclusionary politics.

of health allows us to “squarely identify the political economy underlying the socio-spatial production of suffering” and reorient interventions to not only heal but also address the root causes of harm (Ferguson, 2012).

In order to get to the roots of social determinants like citizenship and structural racism, we cannot ignore the specific infrastructures involving race, political economy, history, and most importantly, borders, even as they often feel beyond the scope of clinical medicine. Borders are both social constructions and material infrastructure, constructed by particular relations of power and under certain political, historical, and economic terms (Agnew, 2008).^v Rather than treating border health as a underpoliticized humanitarian crisis, we want to take a step back to examine some of the histories and forms of power that are ignored or overlooked even in biomedical encounters that recognize social determinants of health. By training a medical gaze to recognize these infrastructural determinants of health, we aim to shift the discourse around the problem of border health in the hopes of creating a different and more radical (i.e. fundamental or rooted) form of healthcare. Rather than accepting unemployment, documentation status or gang violence as social determinants of health, we ask: how exactly have these structural problems come about; how do they continue to be reproduced; what and where are they physically manifested and by whom; and what is the role of the medical professional in effectively addressing them? We hope that in doing so, we can create new horizons for addressing border health that move beyond structural determinants of health (as in providing a forensic evaluation for an asylum claim to secure state rights) and toward infrastructural determinants of health (as in working toward abolition of the border that makes citizenship possible in the first place).

What are the underlying causes of those social pathogenic agents we recognize as documentation status, poverty, or gang violence, among many others? Most of the migrants arriving at the Mexico-US border during the present so-called “crisis” hail from the Northern Triangle countries – El Salvador, Guatemala, and Honduras (Wilson, 2019). In the eyes of many clinicians, Central America is part of a violent, underdeveloped third world that Brown and Black folks leave to seek greater fortunes and possibilities for life in the United States – sometimes because of the imminent risk of death, other times to build better lives for themselves and their families. Indeed, many current immigration debates center on arguments distinguishing economic migrants from asylum seekers – those voluntarily and ‘merely’ seeking an improved life against those involuntarily fleeing imminent death or violent persecution who are somehow more deserving of our humanitarianism or charity (Fassin, 2013).^{vi} Taking a historically attentive and just approach, we refuse both the ahistorical and depoliticized approach that posits Central America as a dangerous Othered land and the kinds of moral distinctions that privilege spectacular violence over structural violence, the fast, eye-catching violence like bloody death over the slow violence wrought by racial-economic policies, corruption, and undemocratic governance including that of the United States (Gutiérrez-Rodríguez, 2018; Uniacke, 2016).

Both kinds of violence, which often are intertwined and inseparable, bring new patients into our healthcare settings. What is often overlooked is that this structural violence is not merely endemic to a distant, war-ridden Central America, unaffected by American life. It is in many ways a product of our own creation, a predictable effect of decades of undemocratic and extractionist American policy and military interference in the affairs of people that our government sees variously as laborers, terrorists, gang members, and other selective forms of identity.

As part of its Cold War strategy to contain the growth of communism globally and particularly in its own backyard, the US government actively repressed revolutionary struggles that offered the possibility for new and more equal social and political relationships to overturn centuries of Spanish colonial and neocolonial rule. In doing so, they supported authoritarian, right-wing regimes that perpetuated state terror, protecting the economic interests of the racial elites (Booth et al., 2010).

In one sense, it is beyond the scope of this article to present an exhaustive analysis of the structural causes of migration from the Northern Triangle to the US (Abrego, 2019; Paley, 2018; Suro, 2019).^{vii} But in another sense, those roots are precisely what needs to be understood and taken seriously to construct more meaningful and informed approaches to border health. We briefly highlight a few key historical causes usually overlooked in mainstream political and health debates on immigration in the United States, in which poverty and violence are taken as natural rather than as a set of inequalities carefully crafted and reproduced through politico-economic and military policy.

Sandra’s experience highlights ongoing strife in El Salvador, where the gang MS-13 (*La Mara Salvatrucha 13*) has come into focus as a particular target for causing violence leading to forced migration to the US. Gang members, who the Trump administration calls ‘violent animals’ and ‘ruthless criminal aliens’ (White House, 2018a, 2018b, 2020), have been characterized as threats to American lives and used to justify the strengthening of border security, including continued construction of border walls. Indeed, for a clinician hearing about the violence Sandra endured at the hands of her abusive partner, it is easy to fall into the kind of moralized thinking that pathologizes and blames the abuser. While this characterization may be partly true, it privileges a way of seeing violence that identifies its causes as ones of moral failing and race, obscuring that which creates the possibilities for violence in the first place. Analyzed individually instead of infrastructurally, clinicians might be tempted to think of Sandra’s abuser as part of a group of dangerous Brown men invading American borders. But as Steven Osuna (2020) and others point out, this ‘transnational moral panic’ over MS-13 obscures the deeper origins of the gang’s violence. “The seeds of MS-13 are found in oligarchic rule, US imperialism, neoliberal restructuring in the Global South and the struggles for national liberation in Central America, on one hand, and the racialized police repression and neoliberal structuring of Los Angeles, on the other,” Osuna explains (Osuna, 2020; Wolf, 2017; Zilberg, 2011). The story of MS-13 is not simply one of violent LA gang members deported to El Salvador, where they wreaked havoc. It is a story of regulation and control by ruling elites in one country; of poverty manufactured through state-sponsored economic violence, including vastly unequal distributions of land and wealth and neoliberal reforms that maintain the extractive global supply chains that continue to enrich them; and of a repressive US-backed junta that fought a civil war from 1979 to 1992 against the leftist Farabundo Martí National Liberation Front (FMLN), which sought to end the reign of landed oligarchy (Bonner, 1984 and LeoGrande, 1998). It was this war that brought Salvadoran refugees to Los Angeles in the first place, where marginalized youth in a neoliberal city with racist policing became part of a global relative surplus population, and survived through the gang violence of MS-13. Gang violence is structural violence produced by infrastructural inequalities and histories (Osuna, 2020).

As with gang violence, there are multiple layers undergirding and supporting the gender-based violence inflicted on Sandra. One way to conceptualize gender-based violence is to blame the machismo of Brown men like her partner. In Sandra’s case, this approach was ultimately successful within the American legal system, which granted her asylum.

^v Climate change and global remittances (hit hard in the 2008 economic crisis and presumably set to continue to fall in the aftermath of the COVID-19 pandemic) are additional key factors not explored in depth here.

^{vi} Eisenhower administration officials had ties to United Fruit Company and would stand to lose profits in any working class movement.

^{vii} Indeed, the Department of Homeland Security has contracted Israeli firms to supply technologies used at the US-Mexico border – technologies used to maintain Gaza as a prison camp. Jimmy Johnson (2012) has aptly termed this, among other forms of global border security enforcement, the “Palestine-Mexico Border.”

She received protection under the class of gender because of claims of sexism and gender-based violence in her home country – a protection that demonizes Salvadoran society as patriarchal and sexist. This does not deny that patriarchal everyday violence and *de facto* legal impunity for its perpetrators was not part of her experience in El Salvador – indeed, these are enormous challenges for Salvadoran feminist activists. But a more nuanced conceptualization recognizes the reality that gender-based violence does not manifest from nowhere. It is fostered through intersecting forms of violence – including racism, capitalism, and colonialism (Becerra, 2019; CISPES, 2019; Rivera, 2017; Speed, 2019). These and other forms of violence do not disappear once asylum is granted, despite the legal illusion that the United States becomes a redemptive ‘sanctuary’ for an asylee fleeing violence elsewhere. A (white feminist) domestic violence framework that solely focuses on problems of Latin American patriarchy without acknowledging intersectional infrastructure reproduces those infrastructures under labels like “socio-cultural norms” and “country conditions.” (Baranowski et al., 2019).

The infrastructures of violence elsewhere in the Northern Triangle are built with similar histories and policies. In Guatemala, the CIA organized the 1954 overthrow of Jacobo Árbenz Guzmán, a social reformer who had nationalized land belonging to the United Fruit Company (the infamous American corporation that helped inspire the creation of the term ‘banana republic’). While Árbenz’s agrarian reforms offered the possibility for a more equal distribution of land, the coup instead dismantled labor movements through policies favored and supported by US economic interests (Booth et al., 2010).^{viii} The 36-year long civil war in Guatemala that began in 1960 – fought, essentially, over unequal land distribution – saw most of the violence directed against indigenous Mayans who had long been displaced from their land to make way for large cash-crop plantations of the elite minority (Krzmaric, 2016; Manz, 2008; Nelson, 2009; Schelsinger and Kinzer, 2005). The state-sponsored violence led to mass displacement, and continued systems of social exclusion separating elite from the majority meant that while technical peace was declared in 1996, actual war continues as crime, terror, and vigilantism, driving people to leave the country (Manz, 2008).

Honduras has become one of the most violent countries in the world, with people (especially women) thought to be fleeing a mix of gang members, corrupt officials, and drug traffickers who threaten their lives. Again, this mix of structural and physical violence was not ‘natural’ to the country nor was it created in a vacuum. The adoption of neoliberal governance in Honduras in the late 1980s–90s, while strengthening economic interests of the domestic elite and the international capitalist community, weakened an already fragile state infrastructure that could address the problems of poverty facing the majority of its citizens (Booth et al., 2010). Meanwhile, concern over drug trafficking through Honduras into the US has increased mechanisms of social control and extortion for a complicit state and police force through the so-called ‘drug wars’. While gangs and drug traffickers take the blame for most of the visible violence, the violence enacted by economic inequality perpetuated by elites with the state and its security forces goes largely unnoticed (Paley, 2014). Possibilities for systemic democratic change were ended in 2009 when President Mel Zelaya was overthrown by a Honduran elite with the *de facto* blessing of the Obama administration (Pine, 2010).

It would be misleading to attribute all the blame for violence, inequality, and poverty to the US government. Much of the unequal present is in fact owed to the kinds of capitalist, racist elite they historically nurtured in Central America – elite who, since colonial times,

have maintained economic and political hegemony over working classes and peasants in both formal legal and illegal (corrupt) ways. U.S. and Central American elites joined hands in passing the Central American Free Trade Agreement in the mid-2000s, which loosened regulations and protected monopoly rights, contributed to weak labor protections, increased displacement of small farmers off their land, and expanded multinational mining corporations seeking to develop polluting gold projects (Cabezas, 2017; Public Citizen, 2018). Promoted by corporations and other elite proponents as a way to curb migration and create economic prosperity in Central America, this trade liberalization flooded Central America with US subsidized grain, against which small-scale farmers were unable to compete (AFL-CIO, 2015; COHA, 2008; Shaffer and Brenner, 2009). Similar promotion of Special Economic Zones (SEZs) such as that in El Salvador offer foreign corporations tax-free havens in which to operate in exchange for local ‘investment’ and employment (Rodríguez, 2018; Stroumbelis, 2018). The Mesoamerica Integration and Development Project (*Proyecto Mesoamérica*), a project by and for Central American elites, promotes privatization and the interests of transnational capital while displacing people off their land and hurting the environment (Toussaint and Garzón, 2017). Predictably, continued pursuit of market gains by global elites hurt the poor and increase economic instability, leading to more migration. The effects of these policies are compounded by the climate crisis, which has hit the Northern Triangle particularly hard, decimating crops and exacerbating displacement (Ayazi and Elsheikh, 2019; Miller, 2017).

It can seem difficult, even impossible, to reconcile these longer complex histories of politico-economic extraction and elite state control with the narratives and experiences of our patients. But it is vital to do so in order to properly address the root causes of disease, particularly the capitalist and imperialist relations of exploitation and forms of neoliberalism that continue to generate poverty and inequality (Osuna, 2020). Whereas many current approaches to border health tend to individualize and depoliticize illness through a humanitarian approach to the suffering of poor people of color, it is only by paying careful attention to the underlying and ongoing histories beyond the surface that we can begin to properly construct effective treatment. Otherwise we risk offering well-intentioned but ultimately counterproductive forms of care – bandages instead of more appropriate incisions-and-drainages for long simmering and deeply rooted social infections.

1.1. Myths and truths of borders

Central to questioning existing paradigms of border health is to query the idea of the border itself. As social scientists have long pointed out, borders are not natural divisions; rather, they are ideological lines drawn by and through political power, many of them during colonial eras (Alvarez, 1995: 449; Anderson et al., 2009: 6). In a world where we often attempt to assert that everyone is human and deserving of equal treatment, systems of citizenship give those born in certain places what amounts to feudal privilege over others who become classified as ‘illegals’ (Arendt, 1951; Carens, 1987). These privileges often predetermine the forms of health and illness of any individual patient. By enacting hierarchies of citizenship on those who cross them, borders ‘follow’ people even as they try to access healthcare, distinguishing who does and does not have the right to access care (Anderson et al., 2009).

It is perhaps not surprising that in times of global unrest like the present, the powerful attempt to concretize fictitious boundaries into material objects. The expanding global construction of border walls from occupied Palestine to the US-Mexico boundary compounds illness as a central infrastructural determinant of health. It is not only a case that borders themselves produce disease and death, be it through medical apartheid created by the so-called ‘separation barrier’ in the West Bank or from dehydration, sexual assault, or any number of traumas while crossing the Sonoran Desert. But also, they are what Jason de León (2015) has called ‘killing machines’ – intentional and strategic apparatuses of a border industrial complex including the

^{viii} These are among the resources offered by the Los Angeles County Immigrant Youth Toolkit, developed by the Immigrant Youth Task Force and the American Academy of Pediatrics California-2 (AAP CA-2) chapter. Access the toolkit at: <https://aapca2.org/toolkit/>.

Border Patrol that turned a sacred land into a killing field (Montiel, 2017).

The US-Mexico border is a space of surveillance, militarization, and security designed to police a territory once inhabited primarily by indigenous peoples. On the American side, the land now policed was acquired partly on the basis of a white supremacist imperialist doctrine (Manifest Destiny) that suggested that it was God's will that colonists violently kill or displace indigenous peoples and take their land (Dunbar-Ortiz, 2014; Horsman, 1981). The border today is militarized and carceral (i.e. transformed into a war zone with an emphasis on the use of military force, detention, incarceration, and violence). It is the site of a huge security industry full of 'smart walls', drones, armored vehicles designed to prevent the movement of poor people of color – an industry that thrives not only in the US, but in Europe, Israel, and elsewhere (Johnson, 2012).^{ix} The US exports its border patrol apparatus across the world, including further south to both the Mexico-Guatemala and Guatemala-Honduras borders. The system – which Jones (2016) calls a 'global border regime' – is designed to limit the right to movement to a wealthy global elite together with its capital (Miller, 2017, 2019). Built on violence, borders suppress and restrict the life of the vast majority of the world's humans, thereby creating a global apartheid. As such, borders are inherently contradictory to the practice of biomedicine.

While compassion and empathy for the plight of the excluded and marginalized is important, its enactment as border healthcare – as it currently exists – offers merely symptomatic treatment rather than treatment of the underlying disease. As long as clinicians and medical social scientists continue to accept border fictions as border facts, the underlying social pathogens will continue to present new complications. For example, under the systems in which we work, compassionate attention to the social determinants of health of our patients includes providing forensic health evaluations in support of asylum claims. Without denying the strategic necessity of this kind of work, we note that this form of advocacy reinforces and works within the existing framework of legal borders, separating 'legitimate' from 'illegitimate' claims on the basis of demonstrable physical or emotional violence. Through an infrastructural approach to border health, and in place of a tacit acceptance of the modern system of borders, we argue for border abolition as a vital but underused treatment in the repertoire of medical intervention. We want border healthcare to move beyond 'know your rights' campaigns and the filing of asylum or refugee claims; to do more than lead tests and hepatitis screenings; to transcend ESL support and enrollment in Medi-Cal.^x We seek a medical toolkit for border abolition, one that turns off the killing machine. Joining border abolitionists, we call for the defunding of Border Patrol, the destruction of border walls, the dissolution of ICE, and the closing of detention facilities, among other treatments, as new medical remedies for diseases born of borders.

2. Towards the health of border abolition

To undo rather than to mitigate the effects of existing infrastructure, border health advocates must broaden their gaze to embrace a framework that is global, shaken free from the constraints imposed by violent borders and traditional professional practice. Rather than acquiesce to the idea of borders as fixed and thus focus interventions on mitigating their harms, we must think about the medical necessity for abolition. Such a framework must be grounded in two truths: people have the right to move and people have the right to stay. Securing the rights to move and to stay demands a call for "Border Abolition" or "No Borders politics" (Achiume, 2019; Anderson et al., 2009). This political framework is

a call for deconstructing not only physical border walls (fences and lines) but also the infrastructure they embody and reinforce. This includes neocolonial capitalist economic structures that extracts and concentrates wealth while displacing surplus populations; military and citizenship systems that violently regulate movement; and legal practices that render bodies illegal and exempt from normative rights. All of these inflict harm on migrants and should therefore be rejected rather than accommodated by border health advocates. While Anderson et al. (2009) and others have made the call for border abolition on ethical and political grounds, here we expand the call to health grounds. The phrase 'border health' like "carceral health", we argue, is a contradiction in terms insofar as borders produce disease among people whose lives are fundamentally shaped by them.

For healthcare providers, the border abolition framework offers unique opportunities but raises several important limitations in practice. On one hand, professional obligation to the health of one's patient demands rejection of health constraints imposed by their illegality – that is to say, advocating for optimal care of undocumented patients whose legal status hinders equal treatment (Ellis and Dugdale, 2019). According to the American Medical Association's (2001) "Declaration of Professional Responsibility: Medicine's Contract with Humanity", physicians should "advocate for social, economic, educational, and political changes that ameliorate suffering and contribute to human well-being."

On the other hand, limiting the clinical focus to the patient in the exam room has a tendency to medicalize, individualize, and depoliticize societal ills. The provider caring for Sandra and her child is well equipped to treat the presenting symptoms with individualized interventions such as fiber and dietary changes but is less prepared to identify the underlying disease whose treatment demands collective transnational infrastructural intervention. Furthermore, the focus on alleviating suffering during the perpetual crisis of the displaced, restricted, and excluded demands interventions within existing institutions despite their limitations. Given Sandra's undocumented status, her provider may scramble to secure funding for her prescriptions, identify charities to provide psychotherapy, and even provide a medical affidavit to support her claims for asylum and status as a legal permanent resident of the United States. While necessary in alleviating Sandra's suffering, this individualized provision of care within institutionalized health service does little to advance the goal of border abolition by reinforcing a tiered system of health care, valorizing desirable asylum-eligible migrants over undesirable migrants, and reifying the power tied to U.S. citizenship. Alternatively, doing nothing would be unjust to those that consider oppression to be morally injurious to the provider rather than an ideological or academic interest. Therefore, for those that seek to provide care at the service of social change, the question becomes "is there another path?"

2.1. Pragmatic vs. liberatory solidarity

Paul Farmer (2003) introduces the idea of pragmatic solidarity as a framework to respond to structural violence through "the rapid deployment of our tools and resources to improve the health and well-being of those who suffer this violence" (220). While laudable in that it "responds to the needs expressed by the people and communities who are living, and often dying, on the edge," pragmatic solidarity, such as that employed by Sandra's provider, often fails to call into question the infrastructure that allow the deployment of services or to declare an upstream goal beyond the proximate suffering (230). In failing to do so, it risks creating a raft of humanism and compassion solely for providers and the select few they are in solidarity with. While necessary, without direction, the effort is ultimately futile amongst tumultuous waves of struggles and violence. Instead we offer "liberatory solidarity" as a response that is both pragmatic in responding to the immediate health sequelae of infrastructural violence while remaining firmly anchored in a border abolitionist agenda.

Lessons and parallels from the prison abolitionist movement offer a

^{ix} One example of this non-reformist reform is the planned closing of the Los Angeles Men's Central Jail.

^x The health movement of the Black Panther Party provides an important historical precedent for when liberatory infrastructural intervention is transformed into counterproductive humanitarian reform.

timely guide for border health activists (Loyd et al., 2012). Border walls, functionally contiguous with prison walls, are physical structures of control that are rooted in infrastructures that exclude, traumatize, and criminalize vulnerable populations. Both are infrastructures through which state violence regulates and criminalizes the surplus populations of globalized capitalism, particularly targeting those made dependent on transnational informal economies for their livelihood in the wake of neoliberal state transformations. If abolition is the absence of police, prisons, and state violence, then it is also the absence of borders, which are maintained by police, prisons, and state violence. Border and prison abolition efforts are one in the same (Walia, 2021).

The aim of the abolitionist movements is not to mitigate the harms of prisons, or even just to eliminate prisons, but to make visible and deconstruct the infrastructures that justify their existence. In the absence of an abolitionist lens, a prison health advocate seeking pragmatic solidarity may advocate for increased mental health resources, staffing and clinical space for incarcerated patients. This intended solidarity may lead to the construction of a new high-quality medical prison, unwittingly supporting expansion and retrenchment of the prison industrial complex. Prison abolitionists refer to this as a reformist-reform, one that mitigates proximate suffering while reinforcing existing infrastructure (Critical Resistance, 2020).

Alternatively, a prison health advocate using an abolitionist lens and working in partnership with broader organizing efforts would highlight the historical role of policing and incarceration in targeting poor communities of color, the homeless, mentally ill, and those with substance use disorders – a role that effectively produces and reproduces poverty and illness. Prison health abolitionists hold mental health to be inherently incompatible with incarceration. Instead of calling for improved carceral care, they partner with patients, families, and advocates to call for available prison funds to be diverted to community-based systems of care and bail and sentencing reforms – thereby unraveling the system that underpins the need for the prison. This “liberatory solidarity” represents a non-reformist reform, or one that advances the abolitionist agenda, and has proven to be successful (Clayton-Johnson, 2021).^{xi}

Drawing on the example of the liberatory solidarity of prison abolitionists, we outline a practice of “liberatory solidarity” for border health advocates striving for border abolition. First, health providers and public health experts should ground border health curricula in a border abolition framework and include infrastructural analyses to broaden the lens of providers in clinical encounters and border health advocacy. Speaking about displacement without these analyses depoliticizes the forces of displacement and risks transforming opportunities for liberatory infrastructural intervention into potentially counterproductive humanitarian reforms (See Nelson, 2011).^{xii} Examples of such reformist-reforms may include increased investment in law enforcement and border patrol to support monitoring and policing for victims of trafficking, or USAID funding for anti-violence programs in Central America that support police militarization and neoliberal political parties (Goodfriend, 2017).

Incorporation of border abolition into medical and public health education also avoids biases that divide migrants into worthy and unworthy, voluntary and involuntary. This is not to say that valorization and demonization should be replaced by universal sympathy for the migrant victim worthy of salvation by the righteous clinical provider. Instead the dichotomy should be replaced by a recognition of both individuals’ experiences of collective infrastructural failures and their agency in undoing this infrastructure. Recognizing the agency of migrants is central to moving from individualized biomedical interventions to collective organizing towards border abolition. Recognizing the agency and power of medicine, whiteness, and the US government is

also central to moving from individualized to liberatory practice. Such infrastructural analysis and recognition of agency allows clinical evaluations and documentations to contextualize rather than solely medicalize patients’ subjective complaints, broadening the possibilities for intervention beyond normative biomedical and service-based interventions (i.e. food stamps, transportation, and housing). Such interventions may include development of medical-advocacy partnerships and referrals to community-based organizers and advocates that are actively working to dismantle the political infrastructure undergirding illness. In Sandra’s case, such partnerships would not only facilitate her opportunities to engage infrastructural determinants of health, but also interrogate the limits and contradictions of clinical spaces as hierarchical, individual, and biomedical in focus, and in doing so begin to liberate them. Some examples include evaluation of clinical policies relating to engagement with ICE and other forms of policing, language justice practices, or citizenship-based barriers to care. Such barriers may be invisible to the provider based on their class positionality and acculturation to the healthcare space.

Transitioning health work from clinical spaces to shared spaces offers another form of liberatory solidarity. Clinical support at day labor centers or at immigration organizing agencies are potential examples of spaces that allow care to be provided in a politicized fashion that is at the service of organizing and social movements instead of inadvertently assuming the politics of the often corporate mission of the medical-industrial complex (Waitzkin, 2018). In this way, healthcare can be used to describe unmet medical need not as a deficiency warranting a technocratic intervention but as a symptom of broader structural pathologies such as racism and capitalism (Nelson, 2011). Such efforts promote collective consciousness and help situate what appear as biomedical complaints within their infrastructural context.

Lastly, providers and public health scholars should recognize their complicity with and challenge the global infrastructure that drives displacement. Efforts should be made to highlight the health impacts of oppressive and extractive domestic policies and efforts by border abolitionists to deconstruct physical impediments to movement. US support for military interventions, neoliberal reforms, structural adjustment programs, intellectual property rights agreements such as the TRIPS agreement (Barton, 2004), and export of US modeled privatized health systems are just some examples of such policies. Physicians can play an important role in joining forces with border abolitionists to dismantle economic and political systems that reinforce inequities, including the elimination of cross border medical hierarchies. The efforts of Physicians for Social Responsibility to counter nuclear arms proliferation offers one model for how healthcare providers can use a health framework to influence contentious geopolitical debates. Such advocacy should remain grounded in global grassroots organizing and reject artificial distinctions between domestic and global health that reinforce existing border paradigms, including the academic separation between global health and social medicine. Transnational organizations like the Committee in Solidarity with the People of El Salvador (CISPES) are grounded in local grassroots movements and create opportunities for collective consciousness of and resistance against global infrastructure failures. The tremendous growth of globalized capital and creation of transnational institutions to support it (WTO, IMF, World Bank) has not yet been effectively mirrored by organized globalized institutions to represent the demands of the most vulnerable. Consequently, while capital, corporations, and global supply chains are more organized than ever before, those exploited by such systems often struggle in isolation.

3. Conclusion

When the church decries revolutionary violence, it cannot forget that institutionalized violence also exists, and that the desperate violence of oppressed persons is not overcome with one-sided laws, with weapons, or with superior force. Instead, as the pope says, revolutionary violence must be prevented by courageous self-sacrifice, by giving up many

^{xi} This is not to deny that there are not migrants and community activists who are not also clinicians. However, the majority of US clinicians treating migrants are shielded from this kind of violence.

comforts. As long as there is not greater justice among us, there will always be outbreaks of revolution. The church does not approve or justify blood revolution and cries of hatred. But neither can it condemn them while it sees no attempt to remove the causes that produce that ailment in our society.

–Oscar Romero (1978), Romero (2003).

In the 1970s, in Sandra's home country of El Salvador, Oscar Romero served as Bishop in the rural community of Santiago de María. Romero recognized that the living conditions of his parishioners were inconsistent with his teachings, and that the traditional church had not only failed to acknowledge this but also actively supported the state power that maintained the poverty of his congregation. Thus he recognized that true solidarity with his parishioners demanded transformation of his role within the church. His practice of liberation theology included using his sermons to make plain the contradiction between his teachings and existing social conditions, supporting his parishioners in organizing for economic and political agency, and denouncing the partnership of the church and oppressive state that maintained the desperation of the poor. Together these efforts would not only transform the church but also lay the foundation for a revolution for a more just society. While the revolution would fall short, allowing many of the infrastructural failures to continue to oppress and displace Salvadorans like Sandra today, the example of liberatory solidarity set by Romero and the Salvadoran people remains. For modern border health advocates, liberatory solidarity demands that we continue those struggles by simultaneously serving and working alongside those made vulnerable to create a world without borders and the violent infrastructure they sustain.

To create real liberatory solidarity, providers should acknowledge their privileged role in societies in which individuals with medical degrees are given more voice and seen as moral authorities, and are less likely to be subjected to state-sanctioned violence than community activists or migrants themselves.^{xiii} This positioning, when properly acknowledged, can be utilized to change the narrative on borders and migration and to support efforts to act directly and intentionally against border imperialism. On the patient level, this can be accomplished by reinforcing an individual's right to migration and addressing small structural barriers through resource sharing; on the community and national level, this can take place through intentional activism, direct action and radical education. Now is the time for a medical border abolition that treats both the causes and symptoms of a widespread global sickness.

Declarations of competing interest

None.

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